

Policy on Procedural Sedation

«Hospital Name»

Document control

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Prepared by	Anaesthesia/Sedation In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Procedural sedation is administered consistently across the hospital, by competent staff, with continuous monitoring and equipment ready for a deeper level of sedation than planned.

1. Purpose

This policy explains how «Hospital Name» administers procedural sedation consistently, obtains informed consent, ensures only competent staff administer it, keeps the person monitoring separate from the person performing the procedure, monitors patients during and after sedation, applies discharge criteria, and keeps equipment ready for deeper-than-intended sedation.

This policy does not cover general informed consent procedures or general anaesthesia in detail — those are covered in other hospital policies.

2. Scope

This policy applies to all staff who administer, monitor, or are otherwise involved in procedural sedation at «Hospital Name».

3. Policy standards

«Hospital Name» administers procedural sedation under written guidance that is standardised across the organisation, obtains informed consent covering risks, benefits, and alternatives, uses only competent staff to administer sedation with the monitoring role kept

separate from the procedure role, monitors a defined set of parameters during and after the procedure, applies documented discharge criteria, and keeps emergency equipment ready in case a patient goes into a deeper level of sedation than intended.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not administer procedural sedation without written guidance covering, at minimum, where sedation is given, how orders are written, pre-procedure assessment, during-procedure monitoring, after-procedure monitoring, and discharge/transfer-out criteria.
- Do not administer sedation without a documented pre-sedation assessment.
- Do not administer sedation without informed consent covering risks, benefits, and alternatives, taken by the person administering sedation or a doctor on the sedation team.
- Do not let a technician administer procedural sedation, under any circumstances.
- Where the parenteral route is used, do not let a nurse administer sedation without a doctor's supervision.
- Do not let the same person both perform the procedure and monitor the sedation.
- Do not skip any of the six required intra-procedure monitoring parameters: heart rate, cardiac rhythm, respiratory rate, blood pressure, oxygen saturation, and level of sedation.
- Do not skip any of the five required post-sedation monitoring parameters: heart rate, respiratory rate, blood pressure, oxygen saturation, and level of sedation.
- Do not discharge a patient from the observation/recovery area without a qualified individual applying documented discharge criteria.
- Do not administer sedation in a room without all five emergency equipment items in working order: emergency resuscitation equipment, suction, advanced airway equipment, positive pressure ventilation, and supplemental oxygen.

5. What we do

5.1 Follow written guidance for consistent sedation

Written guidance based on standard treatment guidelines or sound clinical practice governs procedural sedation. At a minimum, it covers six things: identification of the areas where sedation is administered and procedures performed, the mechanism for writing orders, pre-procedure assessment, monitoring during the procedure, monitoring after the procedure, and discharge or transfer-out criteria. A pre-sedation assessment is performed and documented for every patient, evaluating the risk and appropriateness of sedation, with its scope and content based on professional guidelines. Sedation is administered in a standardised way across the whole organisation.

5.2 Obtain informed consent

Informed consent for procedural sedation is taken by the person administering sedation, or by a doctor on the sedation team. The patient is informed of the risks, benefits, and alternatives, and the patient and family are educated about post-procedural pain relief. (Related requirements are covered in the hospital's other policies.)

5.3 Use only competent, trained personnel

Where the parenteral route is used, sedation is administered by a doctor, or by a nurse under a doctor's supervision. A technician never administers sedation, without exception.

5.4 Keep the monitoring role separate from the procedure role

The person monitoring a sedated patient is always different from the person performing the procedure, without exception. The monitoring person is trained to detect abnormalities in monitoring parameters and to recognise apnoea and airway obstruction.

5.5 Monitor patients during the procedure

Intra-procedure monitoring covers, at a minimum, six parameters: heart rate, cardiac rhythm, respiratory rate, blood pressure, oxygen saturation, and level of sedation. Additional parameters may be monitored case by case. Monitored parameters are documented; routine cardiac rhythm need not be documented continuously on a monitor, but any rhythm abnormality is documented.

5.6 Monitor patients after the procedure

After sedation, the patient's vitals are monitored at intervals specified in the Procedural Sedation SOP maintained by the Anaesthesia/Sedation In-Charge, until the patient has fully recovered. At a minimum, this covers five parameters: heart rate, respiratory rate, blood pressure, oxygen saturation, and level of sedation. The extent and duration of monitoring can be adjusted for the complexity of the procedure and the patient's other health conditions. Level of sedation could be monitored using a checklist covering mild, moderate, and deep sedation. Monitoring is documented.

5.7 Apply discharge criteria

Discharge or transfer-out from the observation/recovery area follows criteria «Hospital Name» has developed and documented, in line with physiologic parameters and sound clinical practice. A qualified individual applies these criteria, and that application is documented.

5.8 Keep equipment ready for deeper sedation than intended

The room where procedural sedation is given has, in working order: equipment for emergency resuscitation, suction, advanced airway equipment, positive pressure ventilation, and supplemental oxygen. Emergency equipment and supplies are immediately available and suited to the type of sedation, the patient's age, and the patient's medical condition. A person trained in airway management, or an anaesthesiologist, is available in «Hospital Name» and able to respond quickly to the area.

6. Stop-work authority

Do not start procedural sedation without consent, without the required monitoring in place, or without a competent person qualified to administer it; do not administer sedation with a technician, or without a separate person monitoring, or where the sedation room lacks emergency equipment for a deeper level of sedation than intended.

Stop-work applies to the start of a planned sedation procedure. Where sedation is already under way and a patient's condition changes unexpectedly, the response continues with the best available qualified help while the gap is escalated.

The person who stops tells the Anaesthesia/Sedation In-Charge and the Medical Superintendent the same shift. Refusing to proceed with unsafe sedation is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and appropriately resourced; receives stop-work notifications the same shift.
Anaesthesia/Sedation In-Charge	Oversees the sedation programme; ensures written guidance, competency requirements, role-separation rule, and emergency equipment standards are in place; receives stop-work notifications the same shift.
Doctors administering sedation	Perform and document pre-sedation assessment; obtain informed consent covering risks, benefits, and alternatives; administer sedation (or supervise nurses where parenteral route is used); apply and document discharge criteria.
Monitoring personnel	Maintain role separation from procedure performer at all times; monitor all six intra-procedure and five post-sedation parameters; detect abnormalities, apnoea, and airway obstruction; document monitored parameters.
Nursing staff	May administer parenteral sedation only under direct doctor supervision; never administer sedation independently via the parenteral route; a technician never administers sedation.

8. Quality monitoring

Monitoring area	What is monitored
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Written guidance	Written guidance covering all six required areas in place and current; pre-sedation assessment performed and documented for every patient
Informed consent	Consent records covering risks, benefits, and alternatives; taken by administering doctor or sedation team doctor
Staff competency — no technician	Parenteral sedation administered only by a doctor or nurse under supervision; no technician administers sedation under any circumstances
Role separation	Person monitoring is always different from procedure performer; monitoring person's training in abnormality detection documented
Intra-procedure monitoring	All six parameters monitored and documented for every procedure: heart rate, cardiac rhythm, respiratory rate, blood pressure, oxygen saturation, level of sedation
Post-sedation monitoring	All five parameters monitored at organisation-defined intervals until full recovery: heart rate, respiratory rate, blood pressure, oxygen saturation, level of sedation; monitoring documented
Discharge criteria	Documented discharge/transfer-out criteria applied by a qualified individual; application documented for every patient
Emergency equipment	All five items in working order checked and recorded: emergency resuscitation equipment, suction, advanced airway equipment, positive pressure ventilation, supplemental oxygen

9. Training and staff acknowledgement

All staff who administer, monitor, or are otherwise involved in procedural sedation shall be familiar with the written guidance applicable to their work, including pre-sedation assessment, informed consent requirements, the absolute prohibition on technicians administering sedation, the role-separation rule, the six intra-procedure and five post-sedation monitoring parameters, discharge criteria, and emergency equipment requirements.

I have read the Policy on Procedural Sedation of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____

_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all staff who administer, monitor, or are otherwise involved in procedural sedation at «Hospital Name».

11. Abbreviations

Abbreviation	Meaning
BP	Blood pressure
SpO2	Peripheral oxygen saturation

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.12.a	Commitment*	Section 5.1 addresses written guidance covering at minimum six areas: identification of sedation areas and procedure locations, order-writing mechanism, pre-procedure assessment, intra-procedure monitoring, post-procedure monitoring, and discharge/transfer-out criteria. Also addresses the pre-sedation assessment performed and documented for every patient.
COP.12.b	Commitment	Section 5.2 addresses informed consent for procedural sedation covering risks, benefits, and alternatives, taken by the administering doctor or a sedation team doctor. Also addresses patient/family education on post-procedural pain relief. Stop-work authority (Section 6) also applies to this element.
COP.12.c	Commitment	Section 5.3 addresses

		competency requirements: parenteral sedation by a doctor or nurse under doctor supervision only. States the absolute prohibition: a technician never administers sedation, without exception. Stop-work authority (Section 6) also applies to this element.
COP.12.d	Commitment	Section 5.4 addresses the absolute rule that the monitoring person is always different from the procedure performer, without exception. Also addresses training of the monitoring person in detecting abnormalities, apnoea, and airway obstruction. Stop-work authority (Section 6) also applies to this element.
COP.12.e	Commitment	Section 5.5 addresses intra-procedure monitoring of at minimum six parameters: heart rate, cardiac rhythm, respiratory rate, blood pressure, oxygen saturation, and level of sedation. Notes that routine cardiac rhythm need not be documented continuously, but rhythm abnormalities must be documented.
COP.12.f	Commitment	Section 5.6 addresses post-sedation monitoring of at minimum five parameters: heart rate, respiratory rate, blood pressure, oxygen saturation, and level of sedation. Cardiac rhythm is NOT in the post-sedation list. Monitoring at organisation-defined intervals until full recovery; monitoring documented.
COP.12.g	Commitment*	Section 5.7 addresses discharge/transfer-out criteria developed and

		documented by the organisation, in line with physiologic parameters and sound clinical practice. A qualified individual applies the criteria, and the application is documented.
COP.12.h	Commitment	Section 5.8 addresses the five emergency equipment items that must be in working order in the sedation room: emergency resuscitation equipment, suction, advanced airway equipment, positive pressure ventilation, and supplemental oxygen. Also addresses availability of a trained airway manager or anaesthesiologist able to respond quickly. Stop-work authority (Section 6) also applies to this element.

13. Required Records/Evidence Checklist

Written guidance and pre-sedation assessment

- Written guidance for procedural sedation covering all six minimum areas: sedation areas, order-writing, pre-procedure assessment, intra-procedure monitoring, post-procedure monitoring, discharge/transfer-out criteria.
- Pre-sedation assessment documented for every patient, covering risk and appropriateness of sedation.

Informed consent

- Informed consent records covering risks, benefits, and alternatives for every sedation procedure.
- Evidence that consent was taken by the administering doctor or a sedation team doctor.
- Records of patient/family education on post-procedural pain relief.

Staff competency and role separation

- Evidence that parenteral sedation is administered only by a doctor or a nurse under direct doctor supervision — no technician administers sedation under any circumstances.
- Evidence that the monitoring role is kept separate from the procedure role for every sedation.

- Evidence of monitoring person's training in detecting abnormalities in monitoring parameters and recognising apnoea and airway obstruction.

Intra-procedure monitoring records

- Intra-procedure monitoring records documenting all six parameters: heart rate, cardiac rhythm, respiratory rate, blood pressure, oxygen saturation, and level of sedation.
- Records of any cardiac rhythm abnormalities documented per episode.

Post-sedation monitoring and discharge records

- Post-sedation monitoring records documenting at minimum five parameters: heart rate, respiratory rate, blood pressure, oxygen saturation, and level of sedation — at organisation-defined intervals until full recovery.
- Discharge/transfer-out records showing a qualified individual applied documented criteria; application documented for every patient.

Emergency equipment

- Equipment check records confirming all five items in working order: emergency resuscitation equipment, suction, advanced airway equipment, positive pressure ventilation, and supplemental oxygen.
- Evidence that a trained airway manager or anaesthesiologist is available in «Hospital Name» and able to respond quickly to the sedation area.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.12.
2. Standard treatment guidelines and professional society guidelines on procedural sedation (reference for written guidance, pre-sedation assessment, and monitoring parameters).

Disclaimer

This policy reorganises the supplied COP.12 objective elements and approved plain-language content into policy format. The stop-work text is reproduced exactly as supplied. Mandatory requirements and their stated modal strength have been retained. The six written-guidance areas (COP.12.a), the absolute prohibition on technicians administering sedation (COP.12.c — "without exception"), the absolute role-separation rule (COP.12.d — "always different", "without exception"), the six intra-procedure monitoring parameters (COP.12.e, including cardiac rhythm), the five post-sedation monitoring parameters (COP.12.f — cardiac rhythm is absent from this list), and the five emergency equipment items (COP.12.h) are all preserved exactly. No monitoring interval has been invented for post-sedation monitoring; the interval is organisation-defined.